



AMERICAN OPTOMETRIC ASSOCIATION

February 23, 2026

The Honorable Robert F. Kennedy, Jr.
Secretary
Department of Health and Human Services
200 Independence Avenue, S.W.
Washington, DC 20201

Dear Secretary Kennedy,

The American Optometric Association (AOA) represents more than 50,000 doctors of optometry, optometric professionals, and optometry students. Doctors of optometry take a leading role in patient care with respect to eye health and vision care, as well as general health and wellbeing. As primary health care providers, doctors of optometry have extensive, ongoing training to examine, diagnose, treat, and manage ocular disorders, diseases, and injuries. The AOA serves the needs of the public and health professionals through the provision of evidence-based clinical practice guidelines that promote prevention, identification, treatment, and management strategies for eye and vision conditions/diseases to improve the nation's health. We appreciate the opportunity to submit comments in response to the Department of Health and Human Services Request for Information entitled "Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care" (RIN 0955-AA13).

What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Barriers to adoption of AI exist due to several interconnected challenges. Many AI tools struggle to integrate seamlessly into electronic health record (EHR) systems and established clinical workflows. In addition, there is at times a lack of transparency in how diagnostic or treatment recommendations are generated which raises physician concerns about safety, accountability, and liability. There are also practical barriers that further compound the issue, as some health care practices may lack the necessary digital infrastructure, IT support, and trained personnel to implement and sustain AI solutions effectively.

What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Reimbursement structures should value clinical judgment, oversight, and responsibility physicians retain when using AI-enabled diagnostic or risk-stratification tools. If AI use is not appropriately recognized in payment models, clinicians and health systems lack incentives to invest in implementation, training, and workflow integration.

HHS should encourage care models that ensure AI-identified findings lead to timely follow-up, referral completion, including tracking appropriate doctor to which patients are referred, as well as ongoing treatment. Systems should track whether patients flagged by AI tools actually receive confirmatory evaluation, appropriate intervention, and continued care. A diagnosis or risk flag alone is not the goal. The

true value of AI lies in improving patient outcomes through sustained treatment, care coordination, and disease management. Without mechanisms to close the referral loop, AI-driven identification of disease risks becoming an isolated event rather than a catalyst for better health.

For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Additional resources and information are needed to empower physicians to more quickly vet and verify the utility of AI enabled non-medical devices that exist. However, any guidance, recommendations or evaluations must also be nimble to keep apace with new innovations that are emerging rapidly.

Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

One of the clearest success stories in clinical AI is in diagnostic imaging. In eyecare there is an FDA approved tool that can autonomously identify diabetic retinopathy risk in primary care settings. In eyecare, AI enabled screenings with high sensitivity and specificity may help identify risk, which is a risk that only a comprehensive eye examination can begin to evaluate, mitigate, and/or address. As AI in health care expands, AOA believes that important criteria must be met. AI should be evaluated based on whether it can improve patient outcomes and has high sensitivity and specificity. Systems using AI to identify patient risk of disease must be held accountable to ensure that referral loops are closed. The value of AI cannot be based solely on the accurate identification of disease risk. Value of AI must be assessed based on whether or not patients identified with potential disease risk receive necessary care and follow up.

What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

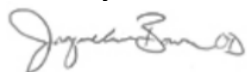
Ongoing education is critical for patients and doctors to understand both the limitations and potential uses of AI in health care. Patients typically want reassurance that:

- Clinicians remain responsible for decisions
- AI recommendations are reviewed, explained, and contextualized
- Care remains individualized, not automated

Patients consistently express that trust resides in the clinician relationship, not in the technology itself.

We appreciate the efforts of HHS to better understand AI adoption challenges and we look forward to continuing to work with HHS to expedite adoption of AI enabled devices that are trusted, valid and can improve patient care. If additional information is needed, please contact Kara Webb at kcwebb@aoa.org

Sincerely,



Jacquie Bowen, O.D.
President, American Optometric Association